



# Physical Restraint and Seclusion in Mental Health Policy

## Scope

| Site                            | Department, Division or Operational Area                                                                                                                                                                                                                                                    | Applicable to                                                     |
|---------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------|
| Royal Perth Bentley Group (RPG) | <i>Authorised</i> mental health settings in Mental Health Division (MHD) and Inpatient units<br><br><i>Non-authorised mental health settings:</i> Transitional Care Unit (TCU) [Bidi Wungen Kaat Centre]), Ward 2K, Mental health Emergency Centre (MHEC) and community mental health (CMH) | Medical, Nursing, Allied Health, External and EMHS Security staff |

## Policy statement

This policy describes the context, enactment, clinical and documentation requirements should physical restraint or seclusion be required to ensure the safety of patients, staff or others.

There are separate processes to be followed in authorised mental health (MH) care settings ([Part A](#) of this document) and in non-authorised MH care settings ([Part B](#) of this document).

Additional considerations apply to inpatients in authorised MH care settings who:

- are under arrest by WA Police or are in the custody of the Department of Correctional Services (refer to **EMHS Prisoners: Management of Policy** and **RPG ED Patients in Police Custody SOP**)
- request patient-initiated discharge (PID) (refer to the **EMHS Discharge Against Medical Advice Policy**)
- require safe transfer to the seclusion room within the East Metropolitan Youth Unit (EMyU). Refer to [Appendix I: Transfer to the EMyU Seclusion Room](#)

The use of mechanical restraints (including handcuffs) within **all MHD inpatient areas is not supported within RPG**.

In the event no other option is available, the situation must be immediately escalated to the MH Executive team.

Should restraint be required for patients within General Health services, refer to relevant RPG General Health policy documents and the [RPG Safe Use and Management of Patient Restraints CPS](#).

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## Definitions

|                                          |                                                                                                                                                                                                                                                                                                                            |
|------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Authorised Hospitals</b>              | Are defined under the <i>Mental Health Act (MHA) 2014</i> . A public hospital or part of a public hospital that has been authorised under the MHA 2014 to receive persons who are mentally ill for examination by a psychiatrist or for admission of involuntary patients who are mentally ill.                            |
| <b>Non-authorised healthcare setting</b> | Any health setting that is not authorised under s.541 of the MHA 2014 e.g. Emergency Department, GP surgery, non authorised general health or MH hospital ward, community clinics.                                                                                                                                         |
| <b>Restraint</b>                         | The restriction of an individual's freedom of movement by physical or mechanical means. <sup>2</sup>                                                                                                                                                                                                                       |
| <b>Mechanical Restraint</b>              | The application of devices (including belts, harnesses, manacles, sheets and straps) on a person's body to restrict their movement. This is to prevent a person from harming themselves or endangering others to ensure the provision of essential medical treatment. <sup>2</sup>                                         |
| <b>Physical Restraint</b>                | The application by healthcare staff of hands-on immobilisation or the physical restriction of a person to prevent the person from harming themselves or endangering others, or to ensure the provision of essential medical treatment. <sup>2</sup>                                                                        |
| <b>Seclusion</b>                         | Confinement of a person who is being provided with treatment at an authorised hospital by leaving the person at any time of the day or night alone in a room or area from which it is not within the patient's control to leave <sup>3</sup>                                                                               |
| <b>Restrictive interventions</b>         | Any practices and interventions that restrict a person's rights, including their freedom to move <sup>4</sup> for the purpose of protecting the person or others from harm.<br>For the purposes of this policy, this includes observation, seclusion, physical restraint, mechanical restraint and rapid tranquillisation. |
| <b>MH Practitioner</b>                   | A MH practitioner can be a psychologist, a nurse, an occupational therapist or a social worker. These health practitioners must have at least three years' experience in the management of people who experience mental illness. <sup>5</sup>                                                                              |
| <b>Personal Support Person</b>           | For this document, personal support person (PSP) includes next of kin (NoK), family, carers, significant others, friends, an Office of the Public Advocate Guardian or Enduring Guardian, or a nominated person, and will be referred to as 'PSP'.                                                                         |

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## Links within document

- [Appendix I: Transfer to the EMyU Seclusion Room](#)
- [Appendix II: Approved forms for restraint in an authorised mental healthcare setting, as per the MHA 2014](#)
- [Appendix III: Approved forms for seclusion in an authorised mental healthcare setting, as per the MHA 2014](#)

## Part A

### Physical restraint and seclusion in an authorised mental healthcare setting

#### General principles

The RPBG MHD is committed to anticipating and reducing the risk of behaviours that might lead to restraint or seclusion being required, within the principles of recovery focused care and in compliance with the MHA 2014, the [Charter of Mental Health Care Principles](#) (MHA 2014) and the **Chief Psychiatrist's Standards for Clinical Care**.

Enaction of physical restraint or seclusion is to be in keeping with the following principles.

- The use of physical interventions may be considered:
  - when there is no other less restrictive option
  - when not doing so would put others at high risk (ie, it is a danger to the patient, staff and others)
  - only when it is deemed proportionate and reasonable in the situation
- Physical restraint and seclusion are emergency interventions, not therapies. They should not be used as part of an individualised care plan
- Suggestion of physical restraint should never be used as an act of coercion or to gain compliance
- The significant risks that the use of physical restraint or seclusion creates for patients with mental illness, their PSPs and staff must be considered. This may include physical harm and/or psychological trauma, loss of dignity, cultural harm, breakdown in therapeutic relationships and death <sup>1</sup>
- Physical restraint may, as a last resort, be required for a patient who is unable to consent but who requires urgent medical and/or psychiatric care. All instances where physical restraint is used must be fully and clearly documented, as per the *Documentation* section in this policy
- Use a multidisciplinary approach to assess and manage care, based on individual needs and setting, with non-pharmacological methods wherever possible

## Physical restraint and seclusion in an authorised mental healthcare setting – General principles cont'd

- Physical restraint will only be considered when **all other practical means of managing the situation, such as de-escalation, distraction or consented medication administration, have failed**
  - any physical restraint carried out by staff should maintain the patient's well-being and be respectful to maintain their dignity at all times
  - any use of physical restraint should be considered from a trauma informed care perspective
  - physical restraint should only be performed as part of a team with appropriate numbers of trained staff and never actioned as an individual
  - sexual safety considerations must be applied during any physical restraint or seclusion event in accordance with the **Chief Psychiatrist's (CP's) Sexual Safety Guidelines of Consumers of Mental Health Services in Western Australia (WA)** and the **Sexual Safety Policy**.

### Ethical Considerations

Restraint or seclusion not only causes acute distress during the experience, but may also exert long-term effects, including self stigmatisation, traumatic memories and symptoms of post-traumatic stress disorder (PTSD) <sup>6, 7</sup> [Level I].

The use of restraints or seclusion in patients with a history of trauma or abuse may lead to re-traumatisation <sup>8</sup> [Level GPP]. Patients may experience increased suffering and staff may be at increased risk of injury during these interventions. For this reason, as a universal precaution, it should be presumed that every person in a treatment setting has been exposed to trauma <sup>9</sup> [Level GPP].

### Pre-Restraint or Seclusion considerations

#### Prevention

- During the patient's admission, the multidisciplinary team (MDT) will ensure use of standardised assessment tools, including the:
  - Statewide Standardised Clinical Documentation (SSCD)
  - Risk assessment and/or *Progressive Risk Assessment* (PRA) if clinically indicated (refer to **Progressive Risk Assessment SOP**)
  - *WA Agitation and Arousal PRN Medication Chart* (refer to the **Medication Chart Policy**)
  - Comprehensive Care documentation
- Consider:
  - any predisposing and precipitating factors
  - the risk of harm to patient, staff or others
- Engage the patient and their PSP in the development of their individualised care plan, including their *What Matters Most* (WMM) form, and identify effective strategies to alleviate their distress
- **Ensure all less restrictive approaches have been attempted first and documented in the patient's integrated notes.**

## Physical restraint and seclusion in an authorised mental healthcare setting – pre-restraint or seclusion considerations – Prevention cont'd

- If staff are concerned regarding a clinical risk-presenting situation or an escalating patient:
  - engage with the patient to identify concerns or unmet needs
  - review possible triggers or physiological causes
  - enact strategies identified with the person or their carers on admission
  - escalate to Shift Co-ordinator and/or Medical Officer (MO) for further review
  - activate Code Black.

### Risk assessment

Before undertaking a physical restraint or seclusion, consideration must be given as to whether it is safe to do so based on:

| <b>Risk assessment of event</b><br>As far as is practicably possible, the following should be considered <b>prior</b> to the application of physical restraint: |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Patient<br>                                                                   | <ul style="list-style-type: none"> <li>• Assessment of the patient, including level of aggression, size, strength, stature, gender</li> <li>• Medical condition, lines and devices attached, pre-existing injury, risk factors for physical restraint related injuries/death.</li> </ul>                                                                                                                                                                                                        |
| Staff<br>                                                                    | <ul style="list-style-type: none"> <li>• The number of appropriately trained staff required to initiate the physical restraint:               <ul style="list-style-type: none"> <li>○ this will vary according to the assessed level of risk. Where possible there should be a gender mix of staffing e.g. females should have female staff present</li> <li>○ identification of a clinical RESTRAINT LEAD responsible for communicating with the team and the patient.</li> </ul> </li> </ul> |
| Environment<br>                                                              | <ul style="list-style-type: none"> <li>• Create space, remove potential weapons</li> <li>• Prepare any additional area if the patient is to be moved from current area</li> <li>• Consider re-directing other patients who may be in close proximity</li> <li>• Prepare medications/charts/documentation</li> <li>• Ensure resuscitation equipment is available.</li> </ul>                                                                                                                     |



## Physical restraint and seclusion in an authorised mental healthcare setting

### Authorising physical Restraint or Seclusion

Authorisation of physical restraint and/or seclusion must be by a MO, mental health practitioner or person in charge of the ward/clinic, as follows:

#### Physical Restraint

- Give verbal authorisation (complete *Form 10A*) or written authorisation (complete *Form 10B*), which allows the patient to be restrained for the shortest duration possible or for up to 30 minutes
  - MO must be informed of physical restraint as soon as practicable and with sufficient time to enable the examination by a medical practitioner within 30 minutes (complete *Form 10E*)
  - The Treating or Duty Psychiatrist must be informed within 30 minutes of the physical restraint and this notification documented on *Form 10C*
  - Complete all physical restraint forms as per the [CP of WA: Restraint Reporting to the Chief Psychiatrist \(authorised hospitals\) Flowchart](#)

#### Seclusion

- Give verbal authorisation (complete *Form 11A*) or written authorisation (complete *Form 11B*) of seclusion
  - MO and the Treating or Duty Psychiatrist must be informed of seclusion as soon as practicable and within two (2) hours (complete *Form 11C*)
  - MO must examine the patient within two (2) hours of being secluded (if seclusion continues) (complete *Form 11E*) and at least every two (2) hours thereafter, or more frequently if clinically indicated
  - MO may make an order extending a seclusion order from the end of the initial period of seclusion for a further period not exceeding two (2) hours
  - A post seclusion medical examination must also be completed within 6 hours of the end of the period of seclusion
  - Complete all seclusion forms as per the as per the [CP of WA: Seclusion reporting to the Chief Psychiatrist \(authorised hospitals\) Flowchart](#).

### During physical Restraint - patient care management

- A patient may be physically restrained in a number of positions depending on their level of aggression, any physiological concerns and the individual risk assessment
- Each position has its own risks of harm to patient and staff. Standing or seated physical restraint should initially be considered, if safe to do so.

Prone physical restraint should be avoided wherever possible and only be used when no other method or position of restraint is feasible <sup>1</sup>. Sudden, unexpected deaths have been reported in such patients (possibly due to asphyxia).

To safely manage the patient during a **prone physical restraint**, ensure **three (3) minutely reviews** are performed, as per *Form 10D - Record of observations made of restrained person*, to minimise patient harm.

## Physical restraint and seclusion in an authorised mental healthcare setting - During physical restraint - patient care management cont'd

There are principles that are applicable for all positions. These include:

1. During physical restraint, under **no circumstances** should direct pressure be applied to the neck, trachea, thorax, abdomen, back or pelvic area <sup>10</sup> [Level GPP].
2. The nose and mouth should never be occluded or covered <sup>11</sup> [Level GPP] unless medically approved e.g. by an oxygen mask.
3. The airway must be continually assessed and the head protected if necessary <sup>12</sup> [Level GPP].
4. There should be no deliberate use of techniques whereby a person is allowed to fall, unsupported, to the floor. When moving a patient from a standing position to prone or supine on the floor, the patient must be supported during the descent and protected from any impact <sup>10</sup> [Level GPP].

### Team roles and responsibilities

Restraints often occur in emergency situations. As soon as practicable, clear roles and responsibilities should be allocated to staff during the restraint with consideration of the following:

| <u>Roles</u>                 | <u>Primary responsibilities</u>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b><u>Lead Clinician</u></b> | <p><b>Note: Only one member of staff is to communicate with the patient at any one time to prevent confusion and reduce stimulation.</b></p> <ul style="list-style-type: none"> <li>• In collaboration with the restraint team, ensure the patient's wellbeing is continually assessed, in particular that the patient's airway, breathing and ability to communicate is not compromised</li> <li>• Communicate with the person being restrained throughout the event in order to continually attempt to de-escalate the situation</li> <li>• Maintain verbal contact between the patient and the restraint team to ensure ongoing assessment of the patient and risk factor for potential/actual adverse outcomes.</li> <li>• Lead the team through the physical intervention process.</li> <li>• Continually assess the patient behaviours with a view to ceasing the intervention as soon as possible</li> <li>• Continual visual physiological monitoring using A-E approach.</li> <li>• Maintain safety of environment and all people present (consider PPE needs).</li> </ul> |
| <b><u>Clinical Team</u></b>  | <ul style="list-style-type: none"> <li>• Follow instructions as per Lead Clinician</li> <li>• Protect the head and neck, as required.</li> <li>• Restraint of arms</li> <li>• Restraint of legs (where requested)</li> <li>• Communication of any concerns to Lead Clinician.</li> <li>• Be aware of position specific risks. Refer to Position specific risks section.</li> <li>• Continual visual physiological monitoring using A-E approach.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |

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|                                     |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|-------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p><b><u>Medical Staff</u></b></p>  | <ul style="list-style-type: none"> <li>• Proceed immediately to location</li> <li>• Receive ISoBAR handover</li> <li>• Complete a risk assessment in collaboration with Shift Coordinator, Clinical Lead and Security</li> <li>• Formulate management plan (short term and long term)</li> <li>• Possible authorisation of PRN medication or restraint               <ul style="list-style-type: none"> <li>○ Refer to <b>Zuclopenthixol Acetate (Clopixol Acuphase) Intramuscular Injection SOP</b></li> </ul> </li> <li>• Monitor physiological status of patient</li> <li>• Post event - review patient condition and document management plan in integrated progress notes and <i>Multidisciplinary Comprehensive Care Plan</i>.</li> </ul>                                                                                                                                                                                             |
| <p><b><u>Scribe</u></b></p>         | <ul style="list-style-type: none"> <li>• The Scribe should document observations of the patient during restraint on <i>Form 10D or 11D</i> and record:               <ul style="list-style-type: none"> <li>○ patency of the airway</li> <li>○ physical condition of the limbs</li> <li>○ record pain score, if any pain stated</li> <li>○ any other details of the restraint, as occurring in real time</li> </ul> </li> <li>• Ensure any medications given are appropriately documented.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| <p><b><u>Other staff</u></b></p>    | <ul style="list-style-type: none"> <li>• Medication – drawing up, checking and administration </li> <li>• ‘Crowd control’ – ensuring other patients and staff that are not required are removed from immediate vicinity</li> <li>• Protecting dignity of the patient</li> <li>• Provide ongoing care for other patients</li> <li>• Calling treating MO and/or Psychiatrist (as appropriate)</li> <li>• Provide handover to arriving response team and direct them to location of incident if other team members are unavailable</li> <li>• Application of physiological monitoring devices when safe to do so.</li> </ul>                                                                                                                                                                                                                                |
| <p><b><u>Security Staff</u></b></p> | <ul style="list-style-type: none"> <li>• Be guided by the Lead Clinician making decision/s involving restraint of the patient.</li> <li>• Must be trained in the appropriate application of physical/ mechanical restraints and have an awareness of the risks posed to staff and patients</li> <li>• In the event that mechanical restraint is required, refer to the <b>Safe Use and Management of Patient Restraints CPS</b>.</li> <li>• If trained, mechanical restraints may be used to:               <ol style="list-style-type: none"> <li>1. Provide lifesaving care and treatment</li> <li>2. Prevent imminent serious harm to the patient</li> <li>3. Prevent harm to others where all other levels of intervention have been unsuccessful</li> </ol> </li> <li>• Compile incident/Sentry report for every incident</li> <li>• Refer to <a href="#">EMHS Security Manual</a> re use of mechanical/physical restraint.</li> </ul> |

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**Physical restraint and seclusion in an authorised mental healthcare setting –  
Team roles and responsibilities cont'd**

If a patient physically deteriorates or reports that they are unable to breathe, an A-E physiological assessment is to occur and identified issues must be addressed. Whilst this assessment is being conducted, continuation of physical restraint is to be risk assessed and reduced or ceased if possible. Refer to **Recognising and Responding to Acute Deterioration Policy**.

MET call to be activated immediately in the event of a medical emergency. Refer to the **Medical Emergencies and Cardiorespiratory Arrest Management CPS**.

**During Seclusion event - Patient care management**

**Clinical practice consideration—sexual safety:**

In accordance with the **OCP's Sexual Safety Guidelines of Consumers of Mental Health Services in WA**, clinicians are to ensure:

- Patients retain their own clothing unless there is a serious clinical contraindication which may compromise safety. These reasons must be documented in the patient's integrated notes
- Patients must not be secluded naked or in underclothes, wherever possible
  - The use of a dignity gown should be considered to minimise risk of harm to self and to maintain patient dignity
  - in circumstances where a patient is unwilling to wear clothes, chooses to remain partially dressed, or it is unsafe to assist the patient to put on clothing, leave appropriate clothing and/or seclusion gown in the room with the patient following risk assessment
- Removing clothing, including underwear, is a last resort and is done on a case-by-case basis. At the point of secluding a patient a risk assessment must be conducted and documented outlining the current risk being mitigated and steps taken to ensure patient dignity is maintained (e.g. covering the patient whilst they remove undergarments and allowing them to change into provided alternatives on their own once staff leave)
- Items removed will be returned immediately upon cessation of seclusion, or earlier if the risk posed by these items is alleviated.

- All hazardous items such as belts, shoes and laces, pens, etc. must be removed.
- Access to dignity gown and blankets to be available on all wards.
- To ensure staff safety, staff should never enter a seclusion room without sufficient staff numbers (minimum of three staff members).
- The MO to ensure completion of all seclusion forms as per the [CP of WA: Seclusion reporting to the Chief Psychiatrist \(Authorised Hospitals\) Flowchart](#).

**Physical restraint and seclusion in an authorised mental healthcare setting – During seclusion event - Patient care management cont'd**

It is imperative clinicians promptly recognise and report any change in the patient's physical and/or mental state or any sudden deterioration. Refer to the **Recognising and Responding to Acute Deterioration Policy** or **Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP**.

- A MH practitioner, Registered Nurse (RN) or Enrolled Nurse (EN) will perform continuous observations
  - Staff must visually sight the patient every 15 minutes and **cannot rely on CCTV cameras alone** as a means of monitoring
  - If CCTV is used, the patient is to be informed at the commencement of seclusion
- MO, RN or EN are to complete 15-minute visual observations, including monitoring of patient's respiratory rate (observing chest rise and fall), movement, alertness, responsiveness and levels of agitation, using Form 11D and *Mental Health Patient Observation and Intentional Rounding Form*
  - the 15-minute clinical observations should be performed via the glass panel in seclusion room door or engaging with the person directly (not via CCTV)
  - an MO must examine the patient at least every two (2) hours and more frequently if clinically indicated. Document findings on *Form 11E*
- Clinicians must ensure that appropriate provision is made for the patient's basic needs, including bedding, clothing, food, drink and toilet facilities
- For the transfer of a patient to the EMyU seclusion room, refer to additional ward-specific requirements in [Appendix I: Transfer to the EMyU Seclusion Room](#)
- A risk assessment must be conducted prior to a decision to revoke, at the expiry of the seclusion order
- MO, MH practitioner or the clinician in charge of the ward or unit can revoke the seclusion order at any time and the patient must be allowed to exit the seclusion area. Complete *Form 11F* (if written order was made or seclusion was extended after the initial 2-hour period).

## Physical restraint and seclusion in an authorised mental healthcare setting cont'd

### Physical Restraint to administer prescribed treatment

- At times physical restraint will be identified as a necessary intervention to support a specific need of a patient i.e. nasogastric tube feeding, urgent medication administration, as authorised by the treating Consultant
- Clinical staff to determine reasons why the patient may not be accepting the treatment and offer a second opinion where no agreement can be reached. Document these reasons in the patient's integrated notes
- Discuss and explain the need for the treatment with the patient, PSP and peer support worker, if relevant
- Any decision to use a planned restraint should be clearly detailed in the patient's progress notes, including the steps taken to reduce or eliminate the need for the restrictive intervention
- The patient and their PSP should be involved where possible in the decision making and planning
- MO, MH practitioner or clinician in charge to ensure completion of *Form 10A/10B* to authorise physical restraint.

### Post Restraint or Seclusion

#### Patient

Check patient wellbeing. An episode of restrictive practice can be traumatic. It is important to acknowledge the element of trauma in this and in past experiences and take necessary steps to manage the consequences.

- Complete a DATIX Clinical Incident Management System (CIMS) only when a restraint or seclusion event has caused harm or injury to the patient
- Document in patient healthcare record.

#### Amendments may be needed to a physical restraint order:

- The MO is to ensure completion of all related forms as per the [CP of WA: Restraint reporting to the Chief Psychiatrist \(Authorised Hospitals\) Flowchart](#).

In accordance with the patient's consent and management plan, clinical staff are to ensure the PSP is informed as soon as practical regarding the use of physical restraint or seclusion, any injuries sustained and when restraint or seclusion was terminated.

- Document in the patient's healthcare record, identifying the PSP informed and the time of the contact.

Senior Clinician to conduct a post-incident debriefing with a patient and/or PSP using a 'lessons-learned' approach to inform future best practice.

- Offer and provide a patient post seclusion or restraint interview
- Refer to the **RFBG Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP**
- Where a patient or a PSP declines a debrief or a decision is taken not to debrief with a patient or PSP, document the reasons in the patient's healthcare record.

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## Physical restraint and seclusion in an authorised mental healthcare setting – Post restraint or seclusion cont'd

### Staff

Senior clinician to perform a staff well-being check immediately post event and following up as required (acknowledging there may be multiple incidences of restraint/seclusion for some acute patients)

- involve the RPBG Wellbeing Team, if needed
- if any staff injury, report via Combined Hazard or Incident Reporting (CHoIR)
- the Work Health and Safety (WHS) department is available to assist and support staff.

Participate in a post-incident debriefing using a 'lessons-learnt' approach to inform future best practice (as above).

### Documentation

The minimum documentation requirements for **every occasion of physical restraint or seclusion** event includes:

- an objective description of the circumstances leading up to the decision to physically restrain or seclude
- documentation in the patient's healthcare record of all less restrictive options that were considered or employed prior to restraint and/or seclusion
- accurately and comprehensively completing and providing a copy of all relevant *MHA 2014 forms* to the:
  - OCP as per relevant flowcharts: [CP of WA: Restraint reporting to the Chief Psychiatrist \(Authorised Hospitals\) Flowchart](#) or [CP of WA: Seclusion reporting to the Chief Psychiatrist \(Authorised Hospitals\) Flowchart](#)
  - the Mentally Impaired Accused Review Board, where applicable
  - Nurse Unit Manager (NUM) or delegate
  - patient, unless it is determined by the MO that to do so may not be in the best interest of the patient or others. MO to document in the patient's healthcare record the reasons for this decision
- in addition to the above OCP forms, document in the patient healthcare record who was involved in the physical restraint and/ or seclusion
- review and update the patient's recovery *Treatment, Support and Discharge Plan (TSDP)*, Psychiatric Service Online Information System (PSOLIS)/ WebPSOLIS (i.e. Alert, service event), and/or WebPAS alert, *Risk Assessment Management Plan (RAMP)* and/or *PRA* (if clinically indicated), *Multidisciplinary Comprehensive Care Plan (MCCP)* and *WWM form*. Include:
  - identifiable factors that may have contributed to the restraint or seclusion and any actions taken to mitigate these factors
  - the patient's post-restraint or seclusion presentation
  - evidence of re-engagement by staff and patient response post restraint or seclusion.

## Physical restraint and seclusion in an authorised mental healthcare setting cont'd

### Restraint or Seclusion Governance

- Weekly MHD Harm Minimisation of Restraint and Seclusion Committee to review of all post- physical restraint or seclusion events
  - Reviewing the privacy, dignity and sexual safety of the patient during the restrictive practice, including provision of clothing, access to toilets and gender of staff to ensure compliance with the [Chief Psychiatrist's Standard for Sexual Safety of Consumers of Mental Health Services](#) (refer to **RPBG Sexual Safety Policy**)
  - the Chair of the Committee to coordinate CCTV random reviews as per [CCTV Monitoring in Mental Health Areas SOP](#)
    - a record of these reviews to be available to the OCP.

### Training requirements

Staff involved in restraint/seclusion are provided with:

- API3 as per the **EMHS Mandatory Training Policy**
- Additional training such as trauma informed/recovery focussed care principles should be integrated into practice to enhance therapeutic alliance
- Security staff to attend security-specific training, as contracted by EMHS

### Compliance monitoring

- Refer to [Part B Compliance monitoring section.](#)



## Part B

### Restraint and seclusion in a non-authorised mental healthcare setting (TCU, Ward 2K, MHEC, CMH)

At RPBG, non-authorised MH settings include:

1. TCU
2. Ward 2K
3. MHEC
4. CMH services.

TCU and CMH staff are not trained in the use of restraint and do not have the resources to safely manage this level of intervention (see specific sections below for further detail)

- the principles of least restrictive practice apply in these settings
- the OCP restraint and seclusion forms (*Forms 10 and 11*) are not applicable in non-authorised MH care settings
- seclusion is not used in TCU, Ward 2K and MHEC
  - there is no designated seclusion room in these areas
  - if seclusion is indicated, patient transfer will be required
- all services must follow the principles and minimum requirements for the safe and lawful practice of restraint in non-authorised MH care settings outlined in the [Department of Health Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#) and practice within the below, Chief Psychiatrist principles.

#### Key Principles

Refer to [Chief Psychiatrist of WA Restrictive Practices Factsheet](#)

- Personal liberty is a fundamental human right
- Interference with a person's liberty is only permitted where it is justified, authorised or excused by law
- When decisions regarding restrictive practices are complex or uncertain, staff are encouraged to escalate to senior clinicians prior to any action being taken
- Patients have a right to refuse care or treatment and leave a hospital or health care facility at any time, unless the patient is incapacitated or presents an immediate risk to themselves, staff or others.

#### Duty of Care

Duty of care does not provide the power to:

- restrain a person
- prevent a voluntary patient from leaving a hospital or healthcare setting
- treat a voluntary patient who refuses treatment.

Duty of care is a legal obligation to take reasonable steps to not cause foreseeable harm to another person or property.

## Restraint and seclusion in a non-authorised mental healthcare setting (TCU, Ward 2K, MHEC, CMH) cont'd

### Doctrine of necessity

- A common law doctrine that requires that:
  - acts are done to avoid immediate risk/harm to the person or others
  - the person carrying out the act did so from an honest and reasonable belief that the individual was placing themselves or others in a situation of immediate risk or harm
  - the act was proportionate to the harm about to occur.

### The Criminal Code (WA)–emergency

- If a person acts in response to what they perceive to be a sudden and extraordinary emergency, and they believe their actions are necessary and reasonable, they may have a defence to any criminal charges relating to that situation.

### When it is unlawful to restrain a person

- Other than in an immediate, grave emergency, there are very few circumstances in which a Health Service Provider (HSP) may lawfully restrain or detain a person in a non-authorised setting
- If a patient is not subject to the MHA 2014 or there are no concerns about the patient's decision-making capacity, they cannot be lawfully restrained or detained. See area-specific processes to follow in each non-authorised MH care setting as outlined below.

### Before seeking to apply a restrictive practice

- All reasonable and least restrictive options must be tried or considered (including de-escalation and distraction practices and processes)
- Clinicians must be satisfied the use of a restrictive practice is absolutely necessary to prevent serious and immediate harm to the patient or another person
- The patient's underlying medical conditions (including pregnancy), past trauma, cultural and social diversity have been considered, where this information is available
- Clinicians are to ensure the use of restrictive practices are applied in a manner that affords the patient dignity and respect.

### Use of Restraint in non-authorised MH care settings

- Restraint may only be used where the following criteria are met prior to engaging in restraint, as per the mandatory Department of Health **Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy**:
  - to prevent significant imminent harm to the patient or others and is considered an intervention of last resort
  - the patient lacks the capacity to make informed decisions about their health care needs at that time, or the patient is of uncertain capacity and is experiencing an acute severe behavioural disturbance and requires an urgent assessment of their capacity and/or treatment of the cause of the behavioural disturbance
  - all reasonable less restrictive options have been attempted or considered to de-escalate or manage the patient's behaviour and/or the imminent or actual risk they pose

#### Physical Restraint and Seclusion in Mental Health Policy

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## Restraint and seclusion in a non-authorised mental healthcare setting - Use of Restraint in non-authorised MH care settings cont'd

- An escalating patient would be reviewed by the medical team and senior nursing staff as per the **Recognising and Responding to Acute Deterioration (RRAD) in Mental State, Cognition and Behaviour SOP** and/or **Recognising and Responding to Acute Deterioration Policy**
  - options for management of the patient must be informed by a risk assessment (RAMP and/or PRA). If imminent risk to patient, staff or others, activate the local service **Code Black Emergency Response Procedure**
    - TCU/CMH: call 000 for urgent Police attendance and ask the patient to leave (Note: on-site security may not be available)
- Shift Coordinator to immediately advise the Psychiatric Registrar/ Resident Medical Officer (RMO) of use of restraint and request **review within 30 mins**:
  - if any physical concerns, consider escalating to Safety All Hours for Everyone (SAFE)/BeSAFE or MO
- Staff should refer to the patient's TSDP, WMM form for information on the patient's triggers, what helps and what doesn't help when they are distressed, to inform their approach to de-escalation and documentation regarding the patient's decision-making capacity
- Patient healthcare record, ISoBAR clinical handover tool, WebPSOLIS /PSOLIS (i.e. Alert, service event, *RAMP*) and/or WebPAS alert and/or *PRA* (if clinically indicated), or *MHA forms* in situ, should be updated
  - complete a DATIX CIMS for a restraint event that causes harm or injury to the patient as well as lodging an OCP Notifiable Incident Report
- *Form 9A—Record of emergency psychiatric treatment* is to be completed by the medical officer ordering treatment if emergency psychiatric treatment is delivered
  - this situation would only occur if the risk of harm to the patient or others is greater by not delivering the treatment compared to delivering the treatment
  - this is regardless of whether the patient is voluntary or on *Form 1A—Referral to a psychiatrist order*
  - the use of restraint does not meet the criteria for a *Form 9A*, neither does sedation alone. This form can only be used for the delivery of treatment for the patient's mental health issues
- During physical restraint, reassess if any high-risk signs identified, i.e. clinical deterioration or any physical injuries resulting from the use of restraint
- Staff to request post-restraint medical review within 30 mins from cessation of restraint (if MO not already present) and document in the patient's healthcare record and/or any relevant safety and quality management system, including Code Black report and restraint monitoring tool
- Refer to the **R PBG Safe Use and Management of Patient Restraint CPS: for General Health areas only**, *Mechanical Restraints* section and [EMHS Security Manual](#), for information regarding approved mechanical restraints at R PBG.

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## Restraint and seclusion in a non-authorised mental healthcare setting (TCU, Ward 2K, MHEC, CMH) cont'd

### Transfer of restrained patient to an authorised healthcare setting

A patient who has been restrained under the emergency provisions outlined above may require transfer from a non-authorised health care setting (eg, Ward 2K) to an authorised healthcare setting (eg, Ward 6).

- If the patient is to be detained for psychiatric assessment, *OCP Forms 1A Referral Order* and *3A Detention Order* must be completed, along with the addition of a WebPSOLIS/PSOLIS 'alert' and 'incident and WebPAS alert. *Form 4A Transport Order* may also need to be completed (Refer to the **EMHS Safe Transportation of MH Consumers Policy**)
- Should any emergency medical treatment, such as medications, be given, a *Form 9A Record of Emergency Psychiatric Treatment* must also be completed.

### Documentation

The minimum documentation requirements for **every** occasion of physical restraint event includes:

- An objective description of the circumstances leading up to the decision to physically restrain, including the patient's mental state, deterioration, associated behaviours and the level and type(s) of risk
- Documentation in the patient's healthcare record of all least restrictive options that were considered or employed prior to restraint and reasons for failure, if applicable
- Completion of documentation in the patient's healthcare record and/or any relevant safety and quality management system, including Code Black report and restraint monitoring tool, signed by MO or Senior Clinician. MO to document:
  - as far as practicable, a contemporaneous record of events during the physical restraint relating to the physical and psychological monitoring of the patient and related staff actions.
  - the type of restraint used and clinical rationale
  - any prior assessment of mental capacity and the outcome of that assessment
  - confirmation that consent to treatment was sought and the outcome
  - the use of security officers, if applicable
  - any physical injuries resulting from the use of restraint or clinical deterioration
  - notification of PSP, guardian and/or enduring guardian, if appropriate
  - any medications offered or administered
  - strategies to reduce time in restraint
  - any multidisciplinary reviews conducted
  - who was involved in the physical restraint and post-restraint medical review outcomes
  - post restraint debriefing offered to the patient and date/time carried out
- Complete Datix CIMS, CHoIR and/or OCP Notifiable Incident reporting

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## Restraint and seclusion in a non-authorised mental healthcare setting (TCU, Ward 2K, MHEC, CMH) – Documentation cont'd

- Review and updating the patient's recovery *TSDP*, *RAMP* and/or *PRA* (if clinically indicated), or *MHA forms* in situ, WebPSOLIS/PSOLIS and/or WebPAS alerts, *MCCP*, *WWM form*. Include:
  - identifiable factors that may have contributed to the restraint and any actions taken to mitigate these factors
  - the patient's post-restraint presentation
  - evidence of re-engagement of and with the patient post restraint.

### TCU (Bidi Wungen Kaat Centre)

- Restraint and/or seclusion are not used in TCU as it is a non-authorised mental recovery and rehabilitation healthcare setting in the community
- All hours: If imminent risk to patient, staff or others, staff should activate the **TCU Code Black Emergency Response Procedure**, call 000 for Police attendance and ask the patient to leave. Security is not available on-site
- In-hours:
  - Early recognition of deterioration is critical to enable early intervention
  - An escalating patient would be reviewed by the medical team and the Clinical Nurse Specialist (CNS)/ Clinical Nurse Manager (CNM)
  - Options for management of the patient must be informed by a risk assessment (*RAMP* and *PRA*)
  - Plan to consider potential deterioration after hours (e.g., plan prepared on a Friday to cover the weekend)
- After hours, on weekends and public holidays: Bentley Health Service (BHS) MH CNS to be advised of any concerns regarding patient deterioration and increase in risk, with escalation of any patient of concern to the DMO
  - Consultant-on-call or Medical Executive-on-call may be contacted by the DMO
  - The Senior Nurse on Call (SNoC) may be contacted to advise of the clinical response of any escalation
- Staff should refer to the patient's *TSDP* and *Client Wellness and Safety Plan* for information on the patient's triggers, what helps and what doesn't help when they are distressed, to inform their approach to de-escalation
- Progress notes, iSoBAR handover, *RAMP*, *PRA* should be updated and Datix CIM, CHoIR and WebPSOLIS/PSOLIS and/or WebPAS alerts completed.

## Restraint and seclusion in a non-authorised mental healthcare setting (TCU, Ward 2K, MHEC, CMH) cont'd

### CMH services (including Safe Haven)

- Restraint and/or seclusion is not used in CMH services as it is a non-authorised MH care setting
- For any behaviours that threaten the imminent safety of patient, staff or others, activate a Code Black immediately
  - contact '000' requesting a Police presence or on-site Security where available and activate a Code Black
  - Safe Haven: contact onsite Security and if response is delayed, call '000' requesting a Police presence
- Complete a Datix CIMS, CHoIR and/or OCP Notifiable Incident Report, WebPSOLIS/PSOLIS and/or WebPAS (ie, alert, service event) and/or *MHA forms* in response to any behaviour or incident and document in the patient's healthcare record. Review and update the patient's recovery *TSDP*, *Safety Plan* and *RAMP*
- Refer to specific MH services *Emergency Procedures* for Midland, City East, Safe Haven, etc, and the **CMH Service Home Visiting Policy**
  - CMHS staff carry duress pendants when on home visits.

### Post Restraint debrief

Lead Clinician to offer and provide a post-incident debriefing with a patient and/or PSP using a 'lessons-learnt' approach to inform future best practice.

- Conduct a post restraint patient interview. Refer to the **R PBG Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP**
- Where a patient or a PSP declines a debrief or a decision is taken not to debrief with a patient or PSP, document the reasons in the patient's healthcare record.

### Staff

Senior clinician to perform a staff well-being check immediately post event and following up as required (acknowledging there may be multiple incidences of restraint/seclusion for some acute patients).

- Involve the R PBG Wellbeing Team, if needed
- Participate in a post-incident debriefing using a 'lessons-learnt' approach to inform future best practice (as above)
  - the WHS department is available to assist and support staff.



## Staff Training

- Staff members (inclusive of security staff) who initiate restraint and those who are responsible for ongoing management of patients under restraint will complete:
  - API2 for CMH and API3 training for all inpatient areas, as per **EMHS Mandatory Policy** and comply with **Department of Health Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy**, including:
    - circumstances that restraint would be deemed lawful or unlawful, appropriate levels of de-escalation, intervention techniques and trauma informed approaches for patient care
    - integrate above training with trauma informed, recovery focussed care' principles. View **Take 5: What is trauma and trauma informed care?**

## Compliance monitoring

In non-authorised MH care settings:

- API 2/3 training compliance rates as monitored by line managers
- Annual report to the Department of Health MH Unit on the rates and types of restraints used (by 31 July)
- Regular review of service configuration and provision to determine if services increase the risk of using restraints and if so, implement strategies to maximise safety and reduce the incidence of restraint
- Regular reporting to improve safety and quality via respective HSP equivalent, patient safety and workforce governance committees, e.g. work health and safety or aggression and violence committees.

In authorised MH care settings:

- at the MHD Safety Quality and Risk meetings.
- API 3 training compliance rates as monitored by line managers
- Summary of findings and improvement actions arising from the MHD Harm Minimisation of Restraint and Seclusion Committee review of restraints or seclusions.

Across all settings:

- Clinical incidents reported in Datix CIMS, CHoIR and/or OCP Notifiable incident reporting resulting from seclusion or restraints, as relevant
- Compliments and Complaints.

## References

1. WACHS. 2020. Mental Health Restraint Policy. Department of Health
2. Chief Psychiatrist of WA. 2024. Chief Psychiatrist's Guideline issued under Sections 547 (3) and 548 of the Mental Health Act 2014: Use of Detention and Restraint in Non-Authorised Healthcare Settings. [Chief Psychiatrist's Guideline on the Use of Detention and Restraint in Non-Authorised Healthcare Settings | Chief Psychiatrist | Government of Western Australia](#)
3. *Mental Health Act 2014*. Western Australia. [Mental Health Act 2014 - \[01-g0-02\].pdf \(legislation.wa.gov.au\)](#)
4. Australian Government. Australian Institute of Health and Welfare (AIHW). 2014. AIHW Restrictive Practices [Restrictive practices \(aihw.gov.au\)](#)
5. Chief Psychiatrist of Western Australian. 2019. Authorised Mental Health Practitioners - Expectations and Requirements. [Authorised Mental Health Practitioners – Expectations and Requirements \(chiefpsychiatrist.wa.gov.au\)](#)
6. Gunawardena R, Smithard DG. 2019. The attitudes towards the use of restraint and restrictive intervention amongst healthcare staff on acute medical and frailty wards – A brief literature review. *Geriatrics*, 2019, 4 (3): 50. [Geriatrics | Free Full-Text | The Attitudes Towards the Use of Restraint and Restrictive Intervention Amongst Healthcare Staff on Acute Medical and Frailty Wards—A Brief Literature Review \(mdpi.com\)](#)
7. Chieze M, Hurst S, Kaiser S, Sentissi O. 2019. Effects of seclusion and restraint in adult psychiatry: A systematic review. *Front Psychiatry*, 2019, 10:491. [Frontiers | Effects of Seclusion and Restraint in Adult Psychiatry: A Systematic Review \(frontiersin.org\)](#)
8. NSW Government. 2020. Restrictive Practices Guidance Environmental Restraint. Department Communities and Justice. [Restrictive Practices Guidance Environmental Restraint | Communities and Justice \(nsw.gov.au\)](#)
9. Tasmanian Government. 2021. Mechanical and Physical restraint. Chief Forensic Psychiatrist Clinical Guideline 10A. [CFP Clinical Guideline 10A Mechanical physical restraint DoHTasmania.pdf \(health.tas.gov.au\)](#)
10. NSW Government. 2024. Restrictive Practices Guidance Physical Restraint. Department Communities and Justice. [Restrictive Practices Guidance Physical Restraint | Communities and Justice \(nsw.gov.au\)](#)
11. Vilke GM. 2020. Restraint Physiology: A review of the literature. *J Forensic Leg Med*. Oct 2020, 75, 102056. [Restraint physiology: A review of the literature - PMC \(nih.gov\)](#)
12. QLD Health. 2020. Physical restraint. Mental Health Act 2016 Chief Psychiatrist Policy. [Physical Restraint \(health.qld.gov.au\)](#)

## Authors/acknowledgements

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## Aboriginal health impact statement and declaration

The voice of the Aboriginal community is reflected in the Physical Restraint and Seclusion in Mental Health Policy through consultation with Aboriginal staff in mental Health, the Aboriginal Health Liaison Officer Team and the Director Aboriginal Health Strategy to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3504).

**Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.**

## Related policy, practice standard, clinical guidelines

### R PBG Policy Hub

- [Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy](#)
- [Safety All Hours For Everyone \(BeSAFE\) /SAFE Policy](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
- [Special and Supportive Observations Policy](#)
- [Prevention and Management of Aggressive Behaviours within MH Settings SOP](#)
- [Patients in Policy Custody \(ED\) SOP](#)
- [EMyU: Youth Custodial Patient Management SOP](#)
- [CMHS Triage to Discharge SOP](#)
- [Mental Health Inpatient Triage to Discharge SOP](#)
- [Community Mental Health Home Visiting SOP](#)
- [CCTV Monitoring in Mental Health Areas SOP](#)
- [Zuclopenthixol Acetate \(Clopixol Acuphase\) Intramuscular Injection SOP](#)
- [Safe Use and Management of Patient Restraints CPS.](#)
- [Medical Emergencies and Cardiorespiratory Arrest Management CPS](#)
- [R PBG Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP](#)
- [Sexual Safety Policy](#)
- [Progressive Risk Assessment SOP](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State, Cognition and Behaviour SOP](#)

**Related policy, practice standard, clinical guidelines cont'd**

EMHS Policy Hub

- [Mandatory Training Policy](#)
- [Chaperone Policy](#)
- [Notification requirements under the WA Mental Health Act 2014 Policy](#)
- [Rights of Patients in Mental Health Services Policy](#)
- [Infection Prevention and Management Policy](#)
- [Management of Prisoners and Detainees Policy](#)
- [Post Incident Management/Critical incident Stress Debriefing Policy](#)
- [Security Services Manual](#)
- [Reducing Restrictive Practices in Mental Health Services Policy](#)
- [Prisoners: Management of Policy](#)
- [Discharge Against Medical Advice Policy](#)
- [Safe Transportation of MH Consumers Policy](#)

WA Department of Health Policy Hub

- [Guidelines for the use of the WA Agitation and Arousal PRN Medication Chart](#)
- [Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#)
- [State-wide Standardised Clinical Documentation for Mental Health Services Policy](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- [Clinical Incident Management Policy](#)
- [Consent to Treatment Policy](#)

Chief Psychiatrist of WA

- [Chief Psychiatrist's \(CP's\) Sexual Safety Guidelines of Consumers of Mental Health Services in Western Australia \(WA\)](#)
- [Chief Psychiatrist of WA Restrictive Practices Factsheet](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Policy for mandatory reporting of notifiable incidents to the Chief psychiatrist](#)
- [CP of WA: Restraint reporting to the Chief Psychiatrist \(Authorised Hospitals\) Flowchart](#)
- [CP of WA: Seclusion reporting to the Chief Psychiatrist \(Authorised Hospitals\) Flowchart](#)

Other

- [Mental Health Act 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Work Health and Safety Act 2020](#)
- Minimising Restraint and Seclusion, – [Take 5](#)
- Trauma Informed Practice – [Take 5](#), What is Trauma Informed Care?

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## Related forms

- *RPBG Physical Restraint Record (General Health and Mental Health (non-authorised healthcare settings))*
- *Multidisciplinary Comprehensive Care Plan*
- *Treatment, Support and Discharge Plan (TSDP)*
- *Risk Assessment Management Plan (RAMP)*
- *MHA 2014 forms*
- *Mental Health What Matters Most*

## Related National Standards

### ACSQHC NSQHS Standards 2<sup>nd</sup> Edition (2021):

Standard 1: Clinical Governance;

Standard 2: Partnering with Consumers;

Standard 5: Comprehensive Care;

Standard 6: Communicating for Safety;

Standard 8: Recognising and Responding to Acute Deterioration

### NSMHS Standards (2010):

Standard 1: Rights and responsibilities;

Standard 2: Safety;

Standard 8 - Governance, leadership and management;

Standard 10: Delivery of care.



## Appendix I: Transfer to the EMyU Seclusion Room

A minimum of **three (3) staff** are required to transfer a patient to the seclusion room i.e. two (2) staff members are to escort the patient into the seclusion room and another staff member is to open and hold the door open during transfer. The patient may be unrestrained (preferred) and the use of physical restraint should be a last resort.

Clinicians are to ensure:

- Continuation of offering diversion strategies to the patient, e.g. the pre-prepared comfort room, increased observation levels or other strategies to be considered on an individual basis
- Doors to South Wing are closed
- Corridors are clear, inclusive of equipment, clinical and non-clinical trolleys (this is a check as they should be kept clear at all times)
- Communication about the transfer to the ward clerk and request that no visitors and/or staff members are permitted through transfer pathway until informed that its open for usual workflow
- There are no patients or visitors present along the transfer pathway, i.e. visitors are to be in the Visitors Room. Patients to be relocated to the South Wing corridor or encouraged to remain in their rooms for the dignity of the patient being transferred and for other patients' safety
- There are no other staff in the seclusion area corridor who are not involved in the transfer
- Laundry and utility room doors are locked at all times (when not in use)
- Doors are opened leading to the North Wing or seclusion room
- Limit access to the seclusion room corridor when seclusion room is in use, i.e. only if necessary to access the treatment or cleaners' rooms.

## Appendix II: Approved Forms for Restraint in an Authorised Mental Healthcare Setting, as per the Mental Health Act 2014

**Physical restraint and seclusion are emergency interventions and should only be used when there is no less restrictive option for the patient.**

**Form completion and clinical observation times are to be used as a minimum.  
Review earlier or more frequently as clinically indicated.**

| Form:      | Title:                                                                                 | Purpose:                                                                                                                                  | Complete form:                          | Expires within: | Can be signed by:                 |
|------------|----------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------|-----------------|-----------------------------------|
| <b>10A</b> | Record of oral authorisation of restraint                                              | To prevent injury or damage to self, others, or property                                                                                  | Prior to or at time of restraint        | 30 minutes      | MO, MHP, person in charge of ward |
| <b>10B</b> | Written bodily restraint order                                                         | To confirm oral authorisation of restraint and/or prevent injury or damage to self, others, or property                                   | Prior to or at time of restraint        | 30 minutes      | MO, MHP, person in charge of ward |
| <b>10C</b> | Record of informing medical practitioner and treating psychiatrist of bodily restraint | To provide a written record of notification to MO or psychiatrist that a person has required restraint                                    | At time of restraint starting           |                 | MO, Psychiatrist                  |
| <b>10D</b> | Record of observations made of restrained person                                       | To ensure that the person being restrained is being monitored to prevent risk of airway obstruction, physical pain, and emotional anguish | Minimum of 30 minutely during restraint |                 | Person observing                  |
| <b>10E</b> | Record of examination of restrained person                                             | To provide a written record of the examination of the restrained person in regards to continuation, variation, or cessation of restraint  | Within 30 minutes of start of restraint |                 | MO                                |
| <b>10F</b> | Variation of bodily restraint order                                                    | To make an order to shorten or vary the bodily restraint                                                                                  | 30 minutes                              |                 | MO, MHP                           |

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 Executive Sponsor: **Mental Health Division**  
 Revision Due: **11/2026**

**Appendix II: Approved Forms for Restraint in an Authorised Mental Healthcare Setting, as per the Mental Health Act 2014 cont'd**

| Form:      | Title:                                           | Purpose:                                                                                                 | Complete form:         | Expires within: | Can be signed by:                                                                 |
|------------|--------------------------------------------------|----------------------------------------------------------------------------------------------------------|------------------------|-----------------|-----------------------------------------------------------------------------------|
| <b>10G</b> | Revocation or expiry of bodily restraint order   | Written record of the revocation or expiration of order to restrain                                      | As soon as practicable |                 | MO, MHP, person in charge of ward. <b>NB. MO or MHP to record expiration only</b> |
| <b>10H</b> | Review of bodily restraint order by psychiatrist | Written record of review of restraint authorisation if restraint has been in place for more than 6 hours | Within 6 hours         |                 | Psychiatrist                                                                      |
| <b>10I</b> | Record of post restraint examination             | Written record of review of restrained person within 6 hours of restraint ending                         | Within 6 hours         |                 | MO                                                                                |

**Physical Restraint and Seclusion in Mental Health Policy**

Version: 3  
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## Appendix III: Approved Forms for Seclusion in an Authorised Mental Healthcare Setting

| Form | Title                                                                                 | Purpose                                                                                                                                 | Time                                                         | Expires | Authorised by?                     |
|------|---------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------|---------|------------------------------------|
| 11A  | Record of oral authorisation of seclusion                                             | To prevent injury or damage to self, others, or property                                                                                | Prior to or at time of restraint                             | 2 hours | MO, MHP, person in charge of ward  |
| 11B  | Written seclusion order                                                               | To confirm oral authorisation of seclusion and/or prevent injury or damage to self, others, or property                                 | Prior to or at time of restraint                             | 2 hours | MO, MHP, person in charge of ward  |
| 11C  | Record of informing medical practitioner and treating psychiatrist of seclusion order | To provide a written record of notification to MO or psychiatrist that a person has required seclusion                                  | As soon as practicable                                       |         | MO, psychiatrist                   |
| 11D  | Record of observations made of secluded person                                        | To provide a written record of visual observations and interventions made during seclusion                                              | Every 15 minutes, or more frequently if clinically indicated |         | MHP, Nurse                         |
| 11E  | Record of examination of secluded person                                              | To provide a written record of the examination of the secluded person in regards to continuation, extension, or revocation of seclusion | Minimum every 2 hours                                        |         | MO                                 |
| 11F  | Revocation or expiry of seclusion                                                     | Written record of the revocation or expiration of seclusion order<br><b>Note: MO or MHP to record expiration only</b>                   | As soon as practicable                                       |         | MO, MHP, person in charge of ward. |
| 11G  | Record of post seclusion examination                                                  | Written record of review of secluded person within 6 hours of seclusion ending                                                          | 6 hours                                                      |         | MO                                 |

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